外傷急症外科操作手冊

1. 急性闌尾炎

- 通常都是術後轉入病房

a. Post-OP order:

Condition: guarded

Diet: NPO / sip water / on diet (視病人術前腸蠕動，手術時間及手術方式而定)

Vital signs: q2h * 2, q4h * 2, then ward routine

IVF: 根據 on diet 情況調整

Wound care: aq-BI QD / wet dressing (視術中 contamination 的情況而定)

Antibiotics: 對 simple appendectomy, 只需要 pre-OP stat dose antibiotics 及術後一天 antibiotics; 對 perforated appendicitis with peritonitis, prolonged antibiotics for 3 to 7 days is sufficient

b. 如果 wound 有紅腫熱痛，或是有 pus discharge，要拆線改成 open care with wet dressing；反之，如果原本就是 open care，若 wound 已經乾淨，可以在 bedside suture 或是用 3M tape approximation

c. 若是病人術後仍高燒不退，或是腹痛未緩解，要考慮是否有 intra-abdominal abscess formation

d. 如果有放 penrose drainage，需監測其 character and amount，如果有逐漸改善可以 advance，且一定要固定好而且確定數量 (核對手術紀錄)

e. 病患有任何可能之合併症，請告知上級住院醫師或主治醫師

2. 急性膽囊炎

可能是術後轉入，或是住院中手術

a. Admission order:

Diagnosis:

Condition: guarded / critical

Diet: depends on patient's condition

Allergy:

Vital signs:

Record drainage: 如果病人已有 PTGBD

IVF : 通常需要 hydration

Lab: liver function, renal function, CBC/DC, PT/ aPTT

Exam: need EKG and CXR if operation is considered

Antibiotics: 視病情而定，通常只需 cefazolin + gentamycin

b. Pre-OP order :

Sign OP and anesthesia permit

NPO since

IVF :

Prepare blood (備血，因為 acute inflammation 開刀容易流血)

OP method: laparoscopic / open cholecystectomy

OP site: abdomen

OP will be performed on

Send patient to OR with chart and X-ray

Fleet enema 1 pc in the morning on OP day

Pre-OP antibiotic stat dose

c. Post-OP order :

與急性闌尾炎類似的術後照護原則

尤其注意膽囊出口之傷口，最容易感染

3. Perforated Peptic Ulcer

- 通常是術後轉入
- 由於 PPI 的療效，目前的治療多採取單純胃穿孔縫合併覆蓋網膜 + 使用 PPI 類藥物

  Post-OP order:

  Condition: guarded

  NPO with NG decompression

  Vital signs: q8h / ward routine

  Record I/O

  IVF: D5S/ D5W 1000ml + KCl 1 amp run full maintain

  Wound care : 視傷口是否已縫合而定 (wet dressing, aq-BI)

  Lab: check electrolytes, Hb/ Hct at coming morning

  Antibiotics with cefazolin + gentamycin
- 術後若病理報告證實有 Helicobacter pyelori，則須服用抗生素治療 +PPI (3-combine therapy)
- Try sip water after 5~7 days，then on soft diet if tolerable。

4. 腸阻塞

- Admission order:

  Condition: guarded

  NPO with NG decompression

  Vital signs: depends on patient's condition

  Record I/O

  IVF: full maintain, with electrolyte supplement

  L/R supplement for NG loss

Prophylactic antibiotics is necessary

Check electrolyte regularly

Follow-up plain film at least q12h

- 密切注意腸音的變化(由 high-pitch 變成 absent bowel sounds)，腹痛的症狀 (是否 progression 或 peritoneal sign 出現)，deterioration of electrolytes and hemodynamics，有無持續排氣；若持續追蹤臨床症狀仍無改善，需考慮手術治療

5. Ischemic Bowel

- 通常也是術後才轉入
- 如果手術中發現大段腸道 poor perfursion，切除會造成 short bowel syndrome (剩餘小腸< 100 公分或切除小腸> 80%)，則考慮對於可能 viable 的腸段保留，只切除已經 gangrene change 的部分。此時不做腸道吻合，待 24~48 小時再次手術觀察存活情形，所謂的 Second look laparotomy.
- Post-OP order :

  大致上與其他腹部手術類似
- 至於已經做腸道吻合的病人，術後必須注意引流管的顏色及流量，是否在吻合處仍然有 ischemic change 而造成 leakage

6. Divercitulitis

- Admission order:

  On the service of:

  Condition: guard

  NPO

  Vital signs as ward routine

  IVF: Full maintain due to NPO

Antibiotic treatment must cover anaerobic

- 觀察腹痛情形，一般在 antibiotic treatment 後疼痛即會緩解。若腹痛持續或是加重，必須考慮 change antibiotic，甚至可能是 perforation 必須 laparotomy
- 病患痊癒後，在門診必須安排 colonoscope examination 或 LGI series，確定有無其他 lesion

7. 胸部鈍傷合併肋骨骨折

- Admission order

  On the service of

  Condition: guarded

  Diet as tolerable

  Vital sign as ward routine

  IVF: depends on intake condition

  Chest permission

  Triflo training

  Follow-up CXR QD * 2 days

  Medication: adequate analgesia for pain control 或 PCA
- 要照 chest X-ray follow-up，注意病患呼吸變化是否有 dyspnea，因為有可能 hemothorax/ pneumothorax 慢慢累積起來

8. 胸管 (chest tube)

- Hemothorax emergent operation indication:

  1.> 1500 ml immediately after tube thoracotomy

  2.> 200 ml/hr blood output，連續二到三小時
- Chest tube 如何拔除?

  a. 每天量< 100 ml serous discharge，兩側呼吸音正常，可考

慮拔除

b. 移除 chest tube 上的活結，如果已打死結，請準備縫合包重新再縫一個 mattress

c. 請病人練習深呼吸，並練習在吸飽氣時憋住氣，此時請助手拔出 chest tube，同時自己把傷口綁緊

d. 切記病人在綁好線後才能正常呼吸

e. 在傷口蓋上 furacin gauge，用布膠密封

f. 照 chest X-ray 追蹤

9. 腹部鈍傷，合併 internal bleeding, with stable hemodynamics

- Admission order:

  Condition: guarded

  NPO

  Vital signs: q8h

  Activity: bed rest

  IVF: full maintain

  Check Hb/ Hct q8h (in ICU)

  Check IAP q8h (in ICU)

  Check 腹圍 q8h (in ICU)

- 密切注意是否有 peritoneal signs 及生命跡象變化 (血壓、心跳、呼吸及意識等)
- 有任何變化須及時反應，請通知總值醫師或主治醫師，如果 hemodynamic unstable，可能需要緊急剖腹探查或血管攝影栓塞治療